

KNOWLEDGE MOBILIZATION TOOLKIT - EVALUATION COMPONENT

Ovarian Cancer Awareness Measurement Tool

Survey instrument for assessing knowledge gaps and evaluating the impact of knowledge mobilization interventions

12

survey items

5 min

completion time

Pre/post design

design

Public +

audiences

Adapted from: Ovarian Cancer Awareness Measure (Simon et al., 2012). Canadian context adapted for AHCP Capstone Toolkit (2026). For use by clinics, researchers, cancer organizations, and public health units.

How to Use This Tool

Who is this for?

This tool has two versions. The **public-facing version (Items 1–10)** is designed for women and members of the general public aged 18 and older. It is suitable for use in community settings, patient waiting rooms, cancer organization websites, and public health campaigns. The **clinician-facing version (Items 1–12)** adds two items assessing clinical knowledge and communication confidence. It is intended for GPs, nurse practitioners, nurses, and other primary care providers, and is best administered as part of a continuing education or KM intervention.

Where does it live?

This tool is designed for both digital and paper delivery. The digital version can be deployed via Qualtrics or a comparable survey platform, linked from the KM Toolkit web page or distributed by email. A print-ready version can be used in clinic waiting rooms, community health events, or alongside the patient fact sheet and symptom action cards.

Who looks at the data?

Responses should be reviewed by the organization or team administering the intervention, such as a clinic coordinator, health educator, public health unit, or cancer organization program lead. Individual responses are anonymous. Aggregate pre- and post-scores are used to assess whether the knowledge mobilization intervention improved awareness, reduced misconceptions, and supported earlier help-seeking intention. Results can also be used to identify which toolkit components to prioritize for a given audience.

Step-by-step: administering the tool

1. Select the appropriate version: Items 1–10 for the general public; Items 1–12 for clinicians.
2. Administer the pre-assessment before participants engage with any toolkit materials.

3. Deliver the knowledge mobilization intervention (e.g. patient fact sheet, myth-busting page).
4. Administer the post-assessment 4 to 6 weeks after the intervention.
5. Compare pre- and post-scores using Section D to identify gaps addressed and gaps that remain.
6. Use the gap flags table in Section D to decide which toolkit components to prioritize for follow-up.

Section A: Design, domains, and scoring

This tool measures five domains across 10 items for the public-facing version, with two additional items in the clinician-facing version. It is designed for pre- and post-deployment: once before a knowledge mobilization intervention and again 4 to 6 weeks after.

Domain	Items	What it measures
Symptom recognition	1, 2	Unaided and aided symptom recall; awareness of persistence as key feature
Screening belief	3, 4	Whether a routine screening programme is believed to exist; understanding of what screening involves
Pap test knowledge	5, 6	What the Pap test detects; whether it provides any information about ovarian cancer
Help-seeking intention	7, 8	Intended action on persistent symptoms; anticipated time to seek care
Barriers to help-seeking	9, 10	Emotional, practical, and informational barriers to presenting with symptoms
Clinical knowledge (clinician version only)	11	Appropriate use of CA-125 testing: diagnostic vs screening
Communication confidence (clinician version only)	12	Self-rated confidence explaining screening absence to patients

Scoring summary

Knowledge items (1 to 6)	Each correct answer = 1 point. Maximum score = 6. Report as percentage correct.
Help-seeking intention (7, 8)	4-point ordinal scale. Higher score = greater intention to seek care promptly.
Barrier items (9, 10)	Each barrier endorsed = 1. Higher total = more barriers. Report descriptively by item.
Pre/post comparison	Subtract pre-score from post-score per domain. Positive change = improvement. Persistent wrong answers flag residual gaps.
Provider items (11, 12)	Item 11: correct/incorrect. Item 12: 4-point confidence scale. Compare pre/post.

Section B: Survey items - public-facing version (Items 1 to 10)

Complete before reading any toolkit materials (pre) and again 4 to 6 weeks later (post).

1 Symptom recognition - select all that apply

Which of the following can be symptoms of ovarian cancer? (Select all that apply)

- ☐ Bloating that is present most days
- ☐ Pelvic or abdominal pain or pressure
- ☐ Difficulty eating or feeling full quickly
- ☐ Needing to urinate more urgently or frequently
- ☐ Unexplained fatigue or low energy
- ☐ Headaches [distractor]
- ☐ None of the above [distractor]

Scoring: 1 point per correct answer (items 1 to 5 above). 0 for headaches or none of the above. Maximum 5 points.

2 Symptom recognition - help-seeking behaviour

If you experienced bloating most days for 2 or more weeks, what would you do?

- ☐ Wait and see if it resolves on its own
- ☐ Mention it at my next routine checkup
- ☐ Book a specific appointment to discuss it
- ☐ Not sure

Scoring: Book a specific appointment = 2; Mention at next routine checkup = 1; Wait and see or not sure = 0.

3 Screening belief - key misconception item

Is there a routine screening test for ovarian cancer available in Canada for women with no symptoms?

- ☐ Yes
- ☐ No
- ☐ I am not sure

Scoring: No = 1 (correct); Yes or not sure = 0. This is the single most important misconception item in the survey.

4 Screening belief - understanding what screening involves

Which of the following is true about ovarian cancer screening in Canada? (Select one)

- ☐ There is a national screening programme using CA-125 blood tests.
- ☐ There is a national screening programme using ultrasound.
- ☐ There is no current routine screening test recommended for average-risk women without symptoms.
- ☐ Family doctors screen for ovarian cancer during routine pelvic exams e.g. Pap test.
- ☐ I am not sure

Scoring: Option 3 = 1 (correct); all others = 0. Distractors target the four most common documented misconceptions.

5 Pap test knowledge - what it screens for

The Pap test (cervical screening) is used to detect:

- ☐ Cervical cancer
- ☐ Ovarian cancer
- ☐ Both cervical and ovarian cancer
- ☐ All gynecologic cancers
- ☐ I am not sure

Scoring: Cervical cancer only = 1 (correct); all others = 0. Addresses the Cooper et al. (2011) and Hawkins et al. (2011) misconception.

6 Pap test knowledge - ovarian cancer information

Can a normal Pap test result tell you whether you have ovarian cancer?

- ☐ Yes
- ☐ No
- ☐ I am not sure

Scoring: No = 1 (correct); Yes or not sure = 0. Core Pap test misconception item -- addresses false reassurance from normal result.

7 Help-seeking intention - response to persistent symptoms

If you had bloating, pelvic pain, and difficulty eating most days for 3 weeks, how soon would you contact your doctor?

- ☐ Within 1 week
- ☐ Within 2 to 3 weeks
- ☐ Within 1 to 2 months
- ☐ I would wait to see if these symptoms resolve
- ☐ I would not seek care

Scoring: Within 1 week = 2; Within 2 to 3 weeks = 3; 1 to 2 months = 1; Wait or not seek = 0.

8 Help-seeking intention - first action with concerns

If you were concerned about ovarian cancer symptoms, what would you most likely do first?

- ☐ I would mention it at my annual checkup
- ☐ I would book a specific appointment to discuss it [preferred action]
- ☐ I would ask to be referred to a specialist
- ☐ I would not be sure what to do

Scoring: Book specific appointment = 2; Ask for referral = 2; Mention at annual checkup = 1; Not sure = 0.

9**Barriers to help-seeking - select all that apply**

**If you had persistent symptoms, which of the following might stop you from seeing a doctor?
(Select all that apply)**

- ☐ I would worry I was wasting my doctor's time
- ☐ I would feel embarrassed discussing these symptoms
- ☐ I would be anxious about what the doctor might find
- ☐ I would not be sure these symptoms were serious enough
- ☐ I would assume my recent normal Pap test meant I was fine
- ☐ I would not want to take time off work or other responsibilities
- ☐ I would not know what to say to my doctor

Scoring: Each barrier endorsed = 1. Higher total = more barriers. Report descriptively by item to identify which barriers are most prevalent. Do not sum into a total score.

10**Barriers to help-seeking - open text**

In your own words, what would make it easier for you to talk to your doctor about symptoms you were concerned about?

Your response:

Section C: Clinician-facing items (Items 11 and 12)

Note: Items 11 and 12 are added only for the clinician-facing version of this survey. They are not included in the standard public version. Administer to GPs, NPs, nurses, residents, and other primary care providers.

11 Clinical knowledge - appropriate use of CA-125

In which of the following situations is CA-125 testing recommended? (Select all that apply)

- ☐ As a routine annual screening test in average-risk women without symptoms
- ☐ To investigate symptoms such as bloating, pelvic pain, or urinary changes
- ☐ After a normal pelvic exam to provide additional reassurance
- ☐ In any woman over 50 years old as a precaution

Scoring: Option 2 only = 1 (correct); any other selection = 0. Addresses the documented provider misconception that CA-125 is appropriate as a routine screening test (Stewart et al., 2012).

12 Communication confidence - explaining screening absence

How confident do you feel explaining to a patient why there is no routine ovarian cancer screening?

- ☐ Not confident at all
- ☐ Somewhat confident
- ☐ Confident
- ☐ Very confident

Scoring: 4-point ordinal scale scored 1 to 4. Report mean score and distribution. Compare pre/post to assess communication confidence gained from the education module.

Section D: Score interpretation and gap identification

Use this guide to interpret pre- and post-results and identify which toolkit components to prioritise.

Score	Level	Interpretation and recommended action
5 to 6	Strong knowledge	Correct on most knowledge items. Still examine help-seeking intention and barrier scores, knowledge alone does not guarantee action.
3 to 4	Partial knowledge	Some key misconceptions present. Review which specific items were incorrect to identify the gap.
0 to 2	Significant gaps	Multiple misconceptions present. Prioritise Pap test clarification, screening absence explanation, and symptom awareness.

Key gap flags: linking survey results to toolkit components

If this item is incorrect...	The gap is...	Prioritise this toolkit component
Item 5 or 6 incorrect	Pap test misconception present	Patient fact sheet Section 1; myth-busting page Section 1
Item 3 or 4 incorrect	Belief that screening programme exists	Patient fact sheet Section 2; myth-busting page Section 2
Item 7 or 8 low score	Delayed or uncertain help-seeking	Patient fact sheet Section 5
Item 9 multiple barriers endorsed	Emotional or practical barriers to care	Clinician conversation guide
Item 11 incorrect (provider)	CA-125 used incorrectly as screening tool	Clinician conversation guide Section 3
Item 12 low score (provider)	Low communication confidence	Clinician conversation guide Sections 2 and 3

Evidence base: Adapted from the validated Ovarian Cancer Awareness Measure (Simon et al., 2012). Items informed by Cooper et al. (2011), Radu et al. (2023), Elshami et al. (2023), Smits et al. (2017), Lockwood-Rayermann et al. (2009), Stewart et al. (2012), and AHCP Capstone scoping review findings (2026).

Deployment: Online via Qualtrics or printed paper version. Pre-assessment at baseline, post-assessment 4 to 6 weeks after intervention, optional 3-month follow-up for retention data.